

Media Information

From: Main Street Radiology

To: [REDACTED]

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Date: 11/12/2025 5:53:51 PM



Bayside 32nd Ave	[REDACTED] Francis Lewis Blvd, Bayside, NY 11358
Bayside 44th Ave	44-01 Francis Lewis Blvd, [REDACTED] NY 11361
Downtown Flushing	136-25 37th Ave, Flushing, NY 11354
Western Queens	72-06 Northern Blvd, Jackson Hts, NY 11372
Glendale	85-14 Cooper Ave, Glendale, NY 11385

Dr. WONG LAP KOON, MD
[REDACTED] SANFORD AVE, SUITE 2A
FLUSHING, NY [REDACTED]

Referrer Phone: [REDACTED]
Name: [REDACTED]
Phone: [REDACTED]
Gender: Female
DOB: [REDACTED]
Patient: [REDACTED]
Exam Date: 10/29/2025 10:15 AM

Acc #: 10207022

Secondary ID: [REDACTED]

Exam Name: [REDACTED]

EXAMS: US BREAST BIOPSY, MA 3D DIGITAL DIAGNOSTIC MAMMOGRAPHY (LEFT BREAST)

HISTORY: 71 years old female with left 10:00, 2 cm from nipple, ill-defined sonographic mass, which corresponds to a left upper central mammographic mass on recent diagnostic evaluation. The patient presents for ultrasound guided core biopsy.

PROCEDURE: Ultrasound guided core biopsy of the left breast.

Pertinent imaging studies and/or documents were reviewed prior to the procedure. The patient identification and procedure were verified. The patient's known allergies, current medications and adverse medication reactions were reviewed prior to the procedure. The procedure was explained to the patient [REDACTED] infection and bleeding, were reviewed. Patient's questions were answered and written informed consent was obtained.

A safety pause was performed immediately prior to the procedure.

PRE PROCEDURE IMAGING: Re-demonstrates the lesion described above.

The patient [REDACTED] obtained. The breast was prepped for the procedure using standard aseptic technique. Local anesthesia was achieved with injection of 7 cc of 1% lidocaine in the skin/superficial tissues and the deeper tissues. The mass was biopsied using a lateral approach, ultrasound guidance and a 14 gauge spring loaded core biopsy needle device. 5 tissue samples were obtained. Adequate lesion sampling was confirmed by orthogonal ultrasound images. The tissue specimens were placed in formalin and submitted to the pathologist [REDACTED] analysis.

A ribbon tissue marker was then deployed at the biopsy site.

Digital post biopsy CC and MLO mammographic views were performed. The breast tissue is heterogeneously dense, which may obscure small masses. Left central inner breast, posterior depth,

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Main Street Radiology

Name: [REDACTED]

Date of Birth: [REDACTED]

ribbon biopsy marker with post biopsy changes is noted. The ribbon biopsy marker corresponds to the irregular mammographic mass, confirming correlation. Just superior to the ribbon clip there is no mammographic asymmetry on MLO projection, which appears relatively stable compared to prior exams.

The patient [REDACTED] of biopsy was held in compression for several minutes and the needle entrance site was closed using Steri strips. Pain control was adequate and the patient was assessed and appropriate for discharge. Home care instructions for this procedure were reviewed with the patient. All patient questions were answered.

IMPRESSION:

Left ultrasound guided core biopsy of the sonographic mass at 10:00, 2 cm from the nipple complete. Ribbon biopsy marker was placed.

Postbiopsy mammogram demonstrates the ribbon clip in the left central inner breast, corresponding to the irregular mammographic mass. Slightly superior, approximately 0.9 cm superiorly, to the ribbon clip on MLO view there is a second mammographic asymmetry, which appears relatively stable compared to prior exams. Given close proximity, this will likely be included in the treatment field.

PATHOLOGY RESULTS:

Invasive ductal carcinoma. Please refer to pathology report for complete results.

Pathology shows malignant findings. The results are concordant with imaging.

RECOMMENDATION:

1. Recommendation: Surgical consultation. Surgery consultation/excision and appropriate medical management is recommended for the left 10:00 (ribbon clip) biopsy-proven invasive ductal carcinoma.

2. Pre and postcontrast breast MRI can be considered to evaluate for extent of disease.

BI-RADS CATEGORY: 6 - Known Malignancy - Biopsy Proven

Attempt was made to contact Dr. WONG LAP KOON MD's office on [REDACTED] 9:28 AM. A copy of this report will be faxed to the doctor's office. A member of our staff will attempt to call the office at a later time.

Electronically signed by: Mrugank Shukla MD [REDACTED] 9:34 AM
Workstation: MSR2-MAMMOPC2

Report Electronically Signed by: Mrugank Shukla MD
Report Electronically Signed on: [REDACTED] 09:34 AM

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Document Information**Other: Outside Correspondence**

bx report

██████████ 00:00

Attached To:
Karen Shih

Source Information

Kayshla Maldonado | Baic Bre Srg
